SERVICE POINT 003 – INTAKE

Goal

Confirm identity and eligibility; complete/standardize background details; screen purpose of visit; verify service and documentation readiness; check payment pathway eligibility; **assign the client to the correct service point** and **handover to Payment** when applicable. {*Confirm eligibility; complete/verify biodata and disability information; capture concise histories and functional screening; signpost to the correct service point; and prepare the payment pathway.*}

Deliverable

Intake Verification & Service Posting Form (with embedded checklists, payment eligibility checks, and auto-routing to Payment / Service Point). {*Intake Assessment & Service Posting Form (web/tablet), integrated with the Health Management Information System (HMIS).*}

Role-Based Access Control

- **Editable:** Intake admin/bio-data (limited), documentation checklist, service screening, payment eligibility, service posting.
- **View-only:** Triage risk flags and vitals; reception check-in; previous visit history.
- **System triggers:** Appointment creation, Payment handoff, SHA (Social Health Authority) referral, deferral checkout, notifications to Customer Care.

Client Flow at Intake

- New Client: Full Intake (all sections editable).
- Old–New Client: Verification Intake (all sections read-only by default; officer clicks Enable Edit per section, selects Reason Code, and edits only missing/outdated fields).
- Returning Client: Skips Intake (already established).

Client Type	Definition / Entry Source	Purpose of Intake	HMIS Route & Actions	Key Differences

New Client	Never registered in the KISE Health Management Information System (HMIS); no Unique Client Identifier (UCI).	To collect all baseline data — biodata, disability details, socio-demographics, and functional screening to guide service posting and payment pathway.	1. Record created at Reception (auto-UCI). 2. Intake opens a blank, fully editable form (all sections enabled). 3. Officer completes full intake: biodata, history, functional screening, and payment eligibility. 4. System validates all mandatory fields before saving. 5. Status → "Cleared for Payment."	Full data entry; all fields editable.
Old–New Client	Already registered in KISE HMIS (from outreach/satellite) but not yet served at Headquarters (HQ).	To verify, standardize, and complete existing records; align clients with HQ protocols and service structures.	1. Record retrieved automatically from the outreach database. 2. System auto-fills biodata, disability, and contact details. 3. Form opens in Verification Mode (Read-Only for already populated form sections). 4. The officer must click "Enable Edit" to unlock already populated fields. 5. Editable fields are already active for missing data or those older than 6 months. 6. Officer reviews and updates disability data, functional screening, and service requests. 7. Status → "Cleared for Payment (admin or cashier)."	Pre-populated record; officer must prompt "Enable Edit" to modify fields. Prevents redundancy and data fatigue.
Returning Client	Previously served at HQ (record active and up to date).	No intake needed — client already profiled and active.	1. Skips the Intake module. 2. Auto routed from Triage → Payment (if therapy follow-up or scheduled service).	Intake button hidden or disabled.

System Logic

Step	Condition / Input	System Action
1	Client record opened in the Intake module	HMIS checks the ClientType flag from Reception.
2	If ClientType = New	Enable **all fields** for data entry.
3	If ClientType = Old–New	Load record from Outreach DB → Lock all fields by default → Allow officer to click "Enable Edit" → Only missing/outdated (>6 months) fields become editable.
4	If ClientType = Returning	Hide Intake module → Auto-route client to Payment.
5	On Save	System merges verified edits into the central database; logs audit trail (User ID, Timestamp, Fields Changed).
6	On Completion	Status auto-updates to "Cleared for Payment"; next module queue (Payment) is triggered.

In Summary

Pathway	Purpose	Data Entry Mode
New Clients	Create new client record; full intake data capture.	Fully editable intake form.
Old–New Clients	Align and verify outreach record; complete missing data.	Locked fields by default; manual unlock required.
Returning Clients	Skip intake; proceed to Payment or Service Point.	Intake module disabled.

Design Benefits:

- Prevents data duplication and fatigue.
- Encourages validation, not re-entry.
- Ensures a single source of truth across Outreach ↔ HQ.
- Supports accountability through audit-trace "Enable Edit" actions.

Intake for Old–New Clients — Edit Policy

Policy

- **Default state:** All sections load **read-only** with existing outreach/satellite data.
- **Edit is allowed for any field**, but **only after the officer explicitly enables edit**.
- **Every edit requires a justification** and is fully **audit-trailed** (who/when/what changed, old→new value, reason).

UI Controls (top sticky bar)

- **[Verify & Continue]** — saves an immutable verification stamp (no changes).

- **[Enable Edit]** (primary) → opens a modal:

    - *"Select sections to edit"* (multi-select list of sections)
    - *Reason for editing* (required; dropdown + optional free-text)
    - *Edit session timeout* (auto 15 min; warning at 13 min; extend button)

- **[Finish Edit & Save]** — runs validation, writes audit log, returns to read-only.
- **[Discard Edits]** — revert to last saved.
- **[Compare]** — side-by-side diff (Old vs New) for this Intake encounter.

Workflow (system logic)

1. Load record → `ClientType=Old–New` ⇒ Already populated field > **read-only**. Blank field > **Editable**
2. Officer reviews → either **Populate (blank fields)**, **Verify & Continue** or **Enable Edit (already populated fields)**.

3. On **Enable Edit**:

    - System locks selected sections for this user (optimistic lock).
    - Fields become editable; change badges appear as you type.

4. On **Finish Edit & Save**:

    - Validate fields; show inline errors.
    - Persist changes + **audit entries per field** (old, new, section, reason).
    - Update "Data Quality Flags" (Resolved / Still Missing).

5. Queue status → **Cleared for Payment** (unless Intake defers).

Audit & Data Integrity

- **Field-level audit**: `{field_id, old_value, new_value, edited_by, edited_at, edit_reason_code, edit_reason_note}`
- **Versioning**: full JSON snapshot per save; "Restore" available to admins.
- **Diff highlighting**: new/changed fields marked with pill "Changed".
- **Reason codes (dropdown)**:
    - Data entry error at outreach
    - Client update (new information)
    - Document verification at HQ
    - Standardization to HQ format
    - Other (specify)

Per-Section Edit Rules — Old–New Clients (Verification Mode)

Section	Default Mode	Editable Fields Logic	How to Unlock	Edit Requirement	Notes / Developer Logic
Header & Context	Read-only		Not editable		Locked: Unique Client Identifier (UCI), Triage Outcome, timestamps.
Client Identification & Contact	Read-only for pre-filled fields, empty fields are auto-editable	If field blank → auto editable. If filled → must "Enable Edit."	"Enable Edit" → Reason Code + short Note.	Name, DOB, and ID changes trigger the *Attach Evidence* modal.	Prevents identity tampering while allowing data completion.
Disability & Registration	Same logic — pre-filled = locked; blanks editable	NCPWD number & Disability Type validated.	"Enable Edit" → Reason + Note.	NCPWD field validates checksum; evidence upload required for number verification.	Dropdown for disability type; "Status" = Seen / Uploaded / Verified.
Socio-Demographic Data	Same logic	Blanks open automatically; filled fields require "Enable Edit."	"Enable Edit" → Reason + Note.	County, Sub-county (pickers), Address (free text), Primary Language, Living arrangement, Marital status.	Each edit event is logged for audit.

Medical & Developmental History	Same logic	Multi-select conditions and medications text is auto-editable if blank.	“Enable Edit” → Reason + Note.	Attach the medical report or card if adding new conditions.	Chronic conditions dropdown; meds free text; attachments optional.
Education / Occupation Status	Same logic	Blank = open; pre-filled = locked until “Enable Edit.”	“Enable Edit” → Reason + Note.	School Type, Level, Attendance, Occupation.	Character limit 100; validated via dropdowns.
Functional Screening Matrix	Same logic	Each blank row is editable; filled → locked until edit enabled.	“Enable Edit” → Reason + Note.	Each “Yes” auto-opens the comment field.	Domains: Vision, Hearing, Communication, Mobility, Learning, Behavior, Self-care, Psychosocial.
Referral & Presenting Concern	Same logic	Blank = editable; pre-filled = locked.	“Enable Edit” → Reason + Note.	Limit 250 characters to prevent narratives.	Text input capped; one statement per visit.
Service Screening / Posting & Availability	Same logic	Dropdowns open; linked data (from dashboard) = read-only.	“Enable Edit” → Reason + Note.	—	Pulls live service availability from Service Availability Dashboard.
Payment Pathway Preview	Same logic	Preview-only; editable only if blank or error flagged.	“Enable Edit” → Reason + Note.	—	View scheme suggestions (SHA, NCPWD, eCitizen, etc.); no billing actions here.
Deferral & Closure	Always editable	—	Dropdown Reason + Next Appointment Date required.	Reason mandatory.	Triggers event intake.deferral_created (with audit).
Time & Audit	Read-only	—	Not editable	—	Auto logs: Intake Start/End, Officer ID, edit events, sync status.

Core Developer Logic

Rule	Implementation Detail
Auto-Editable Blanks	Any field with null or "" auto-renders as editable on load.

Conditional Unlocks	For pre-filled fields, the user must click **Enable Edit** → **Reason + Note modal** before changing.
Save Validation	Cannot save changed data unless Reason + Note is completed (and evidence attached where required).
Autosave & Lock	Auto-saves every 60 seconds. Lock expires after 10 minutes of inactivity or on **Finish Edit**.
Cross-Validation	DOB ↔ Age match; Phone format (E.164); NCPWD checksum.
Audit Trail	Each field change log: field_id, old_value, new_value, user_id, timestamp.

INTAKE SCREENING & ASSESSMENT FORM

A) Header & Context (auto from Reception & Triage)

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Unique Client Identifier (UCI)	Auto (read-only)	Pull from Reception	Locked for all client types.
Client Name / Date of Birth / Sex	Read-only	Pull from Reception	Old–New: change requires "Enable Edit" + reason; name/DOB change requires **evidence attachment**.
Guardian / Phone	Read-only	Pull from Reception	Old–New: unlockable with reason; phone format check.
Triage Status & Risk Flag	Read-only	Pull from Triage	Must be **Cleared for Assessment** to proceed; if Medical Hold/Crisis → block Intake.
Presenting Reason (from Triage)	Read-only	Assessment / First-time Therapy / Medical Report Review / Assistive Technology (AT) consult / Other	Drives posting suggestions.
Intake Start Time / Officer ID	Auto	Set on "Open Intake"	Audit only.

B) Client Identification & Contact Details

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
National ID / Birth Certificate Number	Text	—	Old–New: locked → "Enable Edit" + reason; **evidence required** if changed.
Primary Phone / Alternate Phone	Phone	Kenya phone pattern (+254… or 07…)	Format validation.
Preferred Communication Channel	Dropdown	SMS / Phone Call / Email	Defaults to SMS if blank. If email, ask for their email address.
Consent to Receive SMS	Toggle	Yes / No	Required for reminders.
Primary Address	Text	Free text	—
County/Sub-county / Ward	Cascading pickers	Kenya administrative list	Required; auto loads options.
Landmark / Directions	Text	—	Optional but recommended.

Client Type Logic:

- **New:** All editable.
- **Old–New:** Read-only → **Enable Edit** per field group (reason required). Unpopulated fields are **editable by default**.

C) Disability and Registration Information

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Is disability known?		Yes/No	
Disability Category (tick all that apply)	Multi-select	Hearing / Visual / Physical / Intellectual / Autism / Multiple / Other (specify)	Required if known.
Onset	Radio	Congenital / Acquired / Unknown	—
Level of Functioning	Dropdown	Mild / Moderate / Severe / Profound	Clinician judgement (Intake).
National Council for Persons with Disabilities (NCPWD) Registration	Radio	Yes / No / Unknown	If **Yes**, capture NCPWD Number.

NCPWD Number	Text	—	**Format & existence check** (checksum if available); status: Seen / Uploaded / Verified.
Evidence Attachments	File upload	ID / Birth Certificate / NCPWD Card	Block save if required evidence is missing *(per change).*

Client Type Logic:

- **New:** Fully editable.
- **Old–New:** Read-only → **Enable Edit** (reason required). Unfilled fields are editable.

In the next sections:

Notes:

- Avoid duplication with Reception, Triage, and Service Points.
- Maintain clarity between "background" vs "clinical reasoning,"
- Allow automatic pre-fill (from earlier modules or past visits), and
- Keep Intake focused on *data verification*, not *clinical interpretation.*

D) Socio-Demographic Data

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Marital Status (adults)	Dropdown	Single / Married / Divorced / Widowed / Other (specify)	Shown if age ≥ 18.
Living Arrangement	Dropdown	With family / Institution / Alone / Other (specify)	—
Household Size	Number	1–30	Range check.
Primary Caregiver (children)	Dropdown	Mother / Father / Guardian / Relative / Sponsor / Other (specify)	Show if age < 18.
Source of Support	Multi-select	Parent / Guardian / Government / NGO / Self / Children's Home / Faith-based / Community group / Other (specify)	Kenya context.
Other Source of Support	Text	—	Only if "Other" is picked.
Primary Language	Dropdown	Kenyan Sign Language / English / Kiswahili / Other (specify)	—

Other Languages	Text	—	Optional.
School Enrolment (children & youth)	Radio	Yes / No	If Yes → show school fields (see Section F).
Accessibility at Home (quick check)	Dropdown	Adequate / Limited / Not adequate / Unknown	Background flag only (no reasoning here).

Client Type Logic:

- New: editable.
- Old–New: verification mode (locked until "Enable Edit" with reason; unchanged fields remain read-only).

E1) Medical & Developmental History (Intake)

- Core Medical Snapshot (all clients)

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Known Medical Conditions	Multi-select + free text	Epilepsy / Cerebral palsy / Sickle cell / Asthma / Diabetes / Hypertension / Heart disease / Stroke / Tuberculosis (current/past) / HIV (client-declared) / None / Other (specify)	Client-reported only.
Current Medications	Text	—	Names & dose if known. Optional (free text)
Allergies – Category	Multi-select	Drug / Food / Environmental / Insect / Latex / Herbal / Chemical (incl. pool chlorine) / None Known / Other	Selecting any shows "Specific allergen(s)".

Specific Allergen(s)	Chips multi-select + free text	(Kenya context: Allergies (category) o Drug / Medication Allergy; e.g., Penicillin, Amoxicillin, Cotrimoxazole, Sulfa drugs, Artemether-Lumefantrine, Quinine, Ibuprofen, Diclofenac, Paracetamol, Metronidazole, Ranitidine, Omeprazole, Local anesthetics (Lidocaine).) o Food Allergy (e.g., Eggs, Milk, Fish, Groundnuts/Peanuts, Tree nuts, Maize, Wheat/Gluten, Soy, Sesame, Mango, Banana, Tomatoes) o Environmental Allergy (e.g., Dust, Pollen, Perfume, Smoke, Mold, Grass) o Insect Bite / Sting Reaction (e.g., Bees, Mosquito, Ants) o Latex Allergy (e.g., Gloves, Balloons, Rubber items) o Herbal / Traditional Remedy Reaction (e.g., Local herbs, traditional brews) o Chemical / Pool Substance: Chlorine, Detergents, Disinfectants, Other – specify o Other – Specify: _____	Always allow "Other (specify)". Include searchable chips; always allow "Other (specify)" text. Localized list reflects common Kenyan exposures but remains editable.
Typical Allergy Reactions	Multi-select	Rash/Itch; Swelling; Wheeze/Shortness of breath; Anaphylaxis symptoms; Nausea/Vomiting; Diarrhoea; Abdominal pain; Dizziness/Fainting; Headache; Other (specify)	—
Reaction Severity	Single-select	Mild / Moderate / Severe / Life-threatening (Anaphylaxis)	Required if any allergen.
Previous Assessments	Multi-select + text	Educational / Psychological / Audiology / Physiotherapy / Occupational Therapy / Speech & Language / Vision / Nutrition / None / Other (specify)	Attach report if available.
Assistive Devices History	Multi-select + text	As in (AT Use) Client uses an assistive device/s? If yes, specify (drop-down list select all that apply;	

E2) Assistive Technology Use section:

i)

Field	Input Type	Drop Options / Source / Logic	Notes
Has current AT?	Radio (required)	Yes / No	If No → skip to "AT Needs & Gaps."
AT Category	Dropdown (required)	Mobility (Manual wheelchair; Walker/rollator; Crutches; Cane; Standing frame; Tricycle), Seating & Positioning (custom seat, cushions, lateral supports, headrest, lap belt, tray), Orthotics/Prosthetics (AFO/KAFO/Spinal brace; Upper limb splint; Prosthesis), Vision (Prescription glasses; Low-vision aids; White cane; Braille slate & stylus; Braille display; Magnifier), Hearing (Behind-the-ear hearing aid; In-the-ear hearing aid; Cochlear implant), Communication (AAC) (Communication board/book; Speech-generating device; Tablet-based AAC app), Learning/Access (Pencil grips; Slant board; Keyguard; Alternative mouse/switch), ADL (Feeding aids; Dressing aids; Toilet/shower chair), Other—specify	Kenya-appropriate list; always include Other—specify.
Source of Device	Dropdown	Purchased (private), NCPWD grant, Donor/NGO (e.g., Rotary, Hope Mobility), Public facility, KISE workshop, School, Other—specify	Enables funder reports. Kenya-Context (for Donor/NGO, Public facilities, Schools) Dropdown Libraries
Date Issued (approx.)	Date	—	"Month/Year" is acceptable.
Fit & Comfort	Dropdown (required)	Good; Acceptable; Poor (pain/pressure/instability)	If Poor → auto-flag "AT Safety Risk."
Condition	Dropdown (required)	Good; Worn; Broken/parts missing; Battery fault (if powered)	Broken → route to Repair & Maintenance.
Training Received	Multi-select	Device handover only; Basic use; Safety & transfers; Care/maintenance; None	"None" → suggest User Training.
Use Frequency	Dropdown	Daily; Most days; Weekly; Rarely; Not using	"Rarely/Not using" → prompt for barriers.
Main Environment	Multi-select	Home; School; Work; Community/transport; Therapy	For service planning.
Barriers to Use	Multi-select	Pain/discomfort; Too heavy; Terrain/access; No spares/repair; Embarrassment/stigma;	If "Repair" or "Consumables" → maintenance referral.

		School/work refusal; Battery/consumables cost; Other—specify	
Repairs/review needed	Checkbox + text	Yes (describe) / No	Creates AT Repair ticket draft.
Safety Concerns	Multi-select (red flag)	Pressure areas/sores; Fall risk; Skin breakdown; Hearing/auditory discomfort; Vision strain; Other—specify	Any selected → "AT Safety Risk" pill.
Client/Caregiver Satisfaction	Likert 1–5	1 (very dissatisfied) … 5 (very satisfied)	Optional quick outcome.

ii) Previous AT (if any)

Field	**Input Type**	**Drop Options / Logic**	**Notes**
Any previous AT tried?	Radio	Yes / No	If yes, show a brief grid.
Previous AT Grid (repeatable rows)	Mini-table	Category: Device name; Year; Outcome: Helpful / Not helpful / Outgrown / Broken / Lost; Reason	Keeps history concise; supports clinical decisions.

Assistive Technology Referral Routing Map (Developer's notes)

AT Category (from Intake)	**Referral Destination / Service Point Name (in case of concern)**	**Responsible Team / Role**
Mobility Aids (wheelchairs, crutches, walkers, tricycles, standing frames)	Occupational Therapy or Physiotherapy Service Point	Occupational Therapist, Physiotherapist
Seating & Positioning Equipment (cushions, lateral supports, trays, custom seating)	Occupational Therapy or Physiotherapy Service Point	Occupational Therapist, Physiotherapist
Orthotics / Prosthetics (braces, splints, prostheses)	Orthotics & Prosthetics or Physiotherapy Service Point	Orthotist/Prosthetist, Physiotherapist
Vision Aids (spectacles, magnifiers, Braille, white canes, low-vision devices)	Vision Assessment Service Point	Vision staff

Hearing Aids / Devices (BTE, ITE, bone-anchored, cochlear processors)	Audiology / Hearing Service Point	Audiologist
Communication (AAC Devices) (communication boards, apps, speech devices)	Speech & Language Therapy Service Point (hearing block)	Speech Therapist
Learning / Educational Access Tools (pencil grips, slant boards, alternative computer access)	Functional Educational Assessment Service Point + Occupational Therapy Service Point	Assessor, OT
Activities of Daily Living (ADL) Aids (feeding utensils, toilet/shower chairs, dressing aids)	Occupational Therapy Service Point	Occupational Therapist

Trigger Condition	Referral Route(s)	Priority Tag	System Action (Auto-Generated)
Mobility device = Poor fit / Broken / Safety risk	→ *Occupational Therapy / Physiotherapy*	High	The system should generate the referral automatically at the end of the Assistive Technology (AT) section once the Intake Officer completes their review. The referral recommendation must then be displayed within the receiving service point's HMIS dashboard (e.g., Occupational Therapy, Physiotherapy, Vision, Hearing, Functional Educational Assessment). When the service provider opens the client's profile, the system should show: • The referral source ('Generated from Intake – AT Section') • The reason for referral (e.g., Poor fit, Unmet AT need, Broken device, etc.) • The priority level and any safety flags. This ensures that the therapist or service provider can immediately view,
Vision device = Broken / Unmet need / Last exam > 12 months	→ *Vision Service Point*	Routine	
Hearing device = Fault / Battery issue / Poor fit	→ *Hearing (Audiology) Service Point*	High if device unusable	
Learning / Access tools = Unmet / Ineffective Education / Workplace Access Barriers	→ *Functional Educational Assessment + Occupational Therapy*	Routine	
Communication device (AAC) = Broken / Unused	→ *Speech & Language Therapy*	High	

ADL aids = Missing / Broken / Poor use	→ *Occupational Therapy*	Routine	acknowledge, and act on the recommendation within their own module."
Any device = Broken / Worn	→ *Service point aligning with the selected AT domain (Hearing, vision, mobility, etc)*	High	
Unmet AT need selected, low satisfaction 1 to 2 (any category)	→ *Service point aligning with the selected AT domain (Hearing, vision, mobility, etc)*	Routine	

Referral Display Logic Block –

Trigger / Event	System Action	Display Location	Developer Notes
Event: Intake Officer completes the AT Section and clicks "Save & Generate Referral"	System auto-creates referral record using mapped rules (AT Category → Service Point → Priority → Reason Code).	Referral record stored under the referrals table; status = "Pending Acknowledgement."	Only fires if at least one AT category is selected AND a referral reason exists.
System Action (Secondary): Display recommendation at receiving service point	Auto-push to assigned service point dashboard (OT, Physio, Vision, Hearing, etc.). Record appears in "New Referrals" list.	HMIS Module → Service Point → Dashboard → Referrals	Should be visible to service point user roles (e.g., OT_ROLE, PT_ROLE, AUDIO_ROLE).
When the service provider opens the client profile	Referral details are displayed as a card at the top of the client record or within the Referrals tab.	Client Profile → Referrals sub-tab (or banner alert on Overview).	Add "View Intake Section" link to open original context.
Acknowledgement by receiving the service point	Upon clicking "Acknowledge Referral", status updates → "In Progress."	Service Point Referral Queue	

Client Type Logic: New editable; Old–New verification mode.

E3) Perinatal & Early Development (auto-show if Age < 19)

Field	Input Type	Drop Options / Logic	Validation / Notes
Pregnancy Complications (reported)	Multi-select + text	None / Hypertension / Diabetes / Infection / Bleeding / Substance exposure / Other (specify)	Caregiver-reported only.
Place of Birth	Dropdown	Health facility / Home / Other	—
Mode of Delivery	Dropdown	Spontaneous vaginal / Assisted (forceps/vacuum) / Caesarean section	—
Gestation at Birth (weeks)	Number	22–44 (or months)	Range check: optional.
Birth Weight (kg)	Number	1.0–6.0	Optional if unknown.
Neonatal Care	Multi-select	None / Oxygen / Incubator / NICU admission / Phototherapy / Other	—
Early Medical Issues	Multi-select	Jaundice / Seizures / Infection / Feeding difficulty / None / Other	—
Developmental Concerns	Multi-select + text	Motor / Language / Social / Cognitive / Feeding / Sleep / Behaviour / None / Other	Quick flags only.

E4) Immunization Snapshot (auto-show if Age < 19)

Field	Input Type	Drop Options / Logic	Notes
EPI (Expanded Programme on Immunization) Card Seen Today?	Radio	Yes / No / Unknown	If Yes → allow photo upload (card).
EPI Status (card or caregiver)	Multi-select	BCG / OPV / Pentavalent / PCV / Rotavirus / Measles-Rubella / Yellow fever (if applicable) / HPV (≥9 yrs) / Tetanus-diphtheria (adolescents)	Kenya EPI context; Intake doesn't interpret.

Missed / Unknown Doses	Radio + text	Yes / No → If Yes, specify which	Background flag for downstream visibility.
Recent illness post-vaccine	Radio + text	Yes / No	Optional note.

E5) Feeding & Nutrition Snapshot

Field	Input Type	Drop Options / Logic	Validation / Notes
Feeding History (Age < 5 auto-show; optional for older if Nutrition selected)	Dropdown	Exclusive breastfeeding (6 months) / Mixed feeding / Formula / Complementary feeding per age / Unknown	Caregiver-reported.
Current Diet & Appetite	Dropdown + text	Good / Fair / Poor (+ brief foods)	—
Feeding/Swallowing Concerns	Multi-select	Sucking difficulty / Swallowing difficulty / Coughing/choking / Texture aversion / None / Other	If any → auto-suggest Speech & Language + Nutrition.
Growth Concern (caregiver view)	Radio	Yes / No	BMI-for-age handled in Triage; Intake only records.

F) Education / Occupation Status

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Current Education Level	Dropdown	None / Early Childhood Development (ECD) / Primary / Secondary /Tertiary / Vocational	Age-sensitive display (children and youth)
School Type	Dropdown	Regular / Special / Integrated/Unit / Homeschool	Harmonized labels.

School / Institution Name	Text	—	Optional.
Grade / Level	Dropdown	CBC-aligned list per chosen level ((PP1–Grade 9, Form 1–4, Diploma, etc.))	Optional.
Attendance Challenges	Radio + text	Yes / No → brief reason	—
Academic Performance Concern	Radio + text	Yes / No → brief reason	—
Employment Status (adults)	Dropdown	Unemployed / Employed / Self-employed / Student / Other (specify)	Show if age ≥ 18.
Occupation / Role	Text	—	Optional. Adults only

Client Type Logic: New editable; Old–New verification mode.

G) Functional Screening Matrix (concise; guides posting)

Age Band 0–6 Months

Domain	**Question**	**Response Options**	**Referral Trigger / Red Flag**
Hearing	**1. Does the baby startle when there is a loud sound (e.g., door bang, hand clap)?**	**Yes / No / Unsure**	**No → Refer to *Audiology / Hearing Block* (possible early hearing loss).**
	2. Does the baby calm down when you speak or sing to them?	**Yes / No / Unsure**	**No → *Audiology* (possible difficulty hearing familiar voices).**

Vision	**1. Does the baby look at your face or follow a moving object with their eyes?**	**Yes / No / Unsure**	**No → *Vision Block* (possible vision impairment).**
	2. Do the baby's eyes move in the same direction together?	**Yes / No / Unsure**	**No → *Vision Block* (possible squint or weak eye muscles).**
Communication	**1. Does the baby make soft sounds like "aa," "oo," or "gu"?**	**Yes / No / Unsure**	**No → *Speech & Language Therapy / Early Intervention*.**
	2. Does the baby smile when you talk or play with them?	**Yes / No / Unsure**	**No → *Speech & Language* (possible early social-communication concern).**
Mobility – Gross	**1. Does the baby lift their head briefly when lying on the stomach?**	**Yes / No / Unsure**	**No → *Physiotherapy* (possible motor delay).**
	2. Does the baby try to move arms and legs when excited or awake?	**Yes / No / Unsure**	**No → *Physiotherapy* (low muscle tone or weakness).**
Mobility – Fine	**1. Does the baby open and close their hands?**	**Yes / No / Unsure**	**No → *Occupational Therapy*.**
	2. Does the baby try to hold your finger or a small object placed in their hand?	**Yes / No / Unsure**	**No → *OT / Physio* (possible fine-motor concern).**
Cognition / Learning	**1. Does the baby look toward a sound or light that appears suddenly?**	**Yes / No / Unsure**	**No → *Educational Assessment / Psychologist*.**
	2. Does the baby show interest in new faces or objects?	**Yes / No / Unsure**	**No → *Educational / Psych* (reduced curiosity).**
Social / Behavioural	**1. Does the baby smile when someone familiar approaches?**	**Yes / No / Unsure**	**No → *Counselling / Early Childhood Intervention* (poor bonding).**
	2. Does the baby calm when held, fed, or comforted?	**Yes / No / Unsure**	**No → *Counselling / Child Development Service* (emotional regulation concern).**

Red Flags (Immediate Referral)

- **Does not respond to loud sounds or the caregiver's voice.**
- **Does not look at faces or follow movement.**
- **Moves only one side of the body or appears floppy/stiff.**
- **No smile by 3 months.**
- **Eyes wander or seem "blank" most of the time.**
- **Caregiver reports: "Baby doesn't hear me" / "Baby doesn't look at me."**

Age Band 7–12 Months

Domain	Question	Response Options	Referral Trigger / Red Flag
Hearing	1. Does the baby turn their head when someone calls their name or claps?	Yes / No / Unsure	No → *Audiology / Hearing Block*.
	2. Does the baby notice common household sounds (e.g., spoon falling, dog barking, phone ringing)?	Yes / No / Unsure	No → *Audiology* (possible mild hearing loss).
	3. Does the baby stop crying or get excited when hearing a familiar voice or song?	Yes / No / Unsure	No → *Audiology*; flag "no voice recognition response."
Vision	1. Does the baby look toward a toy or person that moves nearby?	Yes / No / Unsure	No → *Vision Block*.
	2. Does the baby try to reach for objects they see?	Yes / No / Unsure	No → *Vision Block* (possible poor tracking or depth perception).
	3. Do both eyes seem to move together and look straight most of the time?	Yes / No / Unsure	No → *Vision / Eye Clinic* (possible squint or lazy eye).
Communication	1. Does the baby make repeated sounds such as "ba," "ma," or "da"?	Yes / No / Unsure	No → *Speech & Language Therapy*.

	2. Does the baby respond when you say "no" or "bye-bye"?	**Yes / No / Unsure**	**No → *Speech & Language / Early Childhood Intervention*.**
	3. Does the baby use different sounds or tone when "talking" to you?	**Yes / No / Unsure**	**No → *Speech & Language* (poor expressive skill development).**
Mobility – Gross	**1. Can the baby sit without support?**	**Yes / No / Unsure**	**No → *Physiotherapy* (delayed motor milestone).**
	2. Does the baby crawl or move around by rolling or shuffling?	**Yes / No / Unsure**	**No → *Physiotherapy*; flag "not mobile by 10 months."**
	3. Does the baby stand while holding furniture or your hands?	**Yes / No / Unsure**	**No → *Physiotherapy* (weakness or poor balance).**
Mobility – Fine	**1. Does the baby pick up small items (e.g., beans, buttons) using thumb and finger?**	**Yes / No / Unsure**	**No → *Occupational Therapy*.**
	2. Does the baby transfer a toy from one hand to the other?	**Yes / No / Unsure**	**No → *OT / Physio* (possible coordination issue).**
	3. Does the baby bang two objects together?	**Yes / No / Unsure**	**No → *OT / Physio*.**
Cognition / Learning	**1. Does the baby look for a toy that falls or hides under a cloth?**	**Yes / No / Unsure**	**No → *Educational Assessment / Psychology*.**
	2. Does the baby show curiosity about new objects or faces?	**Yes / No / Unsure**	**No → *Educational / Psych* (possible delayed awareness).**
	3. Does the baby recognize familiar people (e.g., mother, sibling, nanny)?	**Yes / No / Unsure**	**No → *Educational / Psych* (social-cognitive delay).**
Social / Behavioural	**1. Does the baby enjoy simple hiding or surprise games, like when you cover your face and smile again?**	**Yes / No / Unsure**	**No → *Counselling / Early Intervention* (social play delay).**

	2. Does the baby show fear of strangers or cling to familiar people?	Yes / No / Unsure	No → *Counselling / Psychosocial support* (poor attachment).
	3. Does the baby show interest when others laugh or talk nearby?	Yes / No / Unsure	No → *Counselling / Psych* (social disengagement).

Red Flags (Immediate Referral)

- **Does not turn to name or loud sound by 9 months.**
- **Does not sit or bear weight on legs.**
- **Does not reach or grasp toys.**
- **No babbling or response to speech.**
- **Eyes not aligned, moving separately.**
- **No interest in people or play.**
- **Caregiver says, "He/she doesn't look when I call."**

Age Band 1–2 Years (12–24 Months)

Domain	Question	Response Options	Referral Trigger / Red Flag
Hearing	1. Does the child respond when their name is called from another room or direction?	Yes / No / Unsure	No → *Audiology / Hearing Block* (possible hearing loss).
	2. Does the child enjoy listening to songs or stories?	Yes / No / Unsure	No → *Audiology / Speech & Language*.
	3. Does the child point to common items when named (e.g., cup, ball, shoe)?	Yes / No / Unsure	No → *Audiology / S&L* (poor receptive language).
Vision	1. Does the child notice small items on the ground (e.g., coin, crumb)?	Yes / No / Unsure	No → *Vision Block*.
	2. Does the child bump into furniture or trip often, even in familiar places?	Yes / No / Unsure	Yes → *Vision Block*.

	3. Do the child's eyes appear straight and move together most of the time?	**Yes / No / Unsure**	**No → *Vision Block* (possible squint).**
Communication	**1. Does the child use 10–20 single words (e.g., "mama," "bye," "milk")?**	**Yes / No / Unsure**	**No → *Speech & Language Therapy*.**
	2. Does the child try to combine two words (e.g., "mama come," "want water")?	**Yes / No / Unsure**	**No → *Speech & Language*.**
	3. Does the child follow simple instructions (e.g., "sit down," "come here")?	**Yes / No / Unsure**	**No → *S&L / Early Intervention*.**
Mobility – Gross	**1. Can the child walk alone without holding onto furniture?**	**Yes / No / Unsure**	**No (by 18 months) → *Physiotherapy*.**
	2. Can the child bend down to pick up a toy and stand again without falling?	**Yes / No / Unsure**	**No → *Physiotherapy*.**
	3. Can the child climb onto a low stool or couch?	**Yes / No / Unsure**	**No → *Physiotherapy*.**
Mobility – Fine	**1. Can the child feed themselves using hands or a spoon?**	**Yes / No / Unsure**	**No → *Occupational Therapy*.**
	2. Does the child stack two or more blocks or cups?	**Yes / No / Unsure**	**No → *Occupational Therapy / Early Learning*.**
	3. Can the child turn book pages or pick small objects using fingers?	**Yes / No / Unsure**	**No → *OT / Vision*.**
Cognition / Learning	**1. Does the child copy simple actions like sweeping, stirring, or talking on the phone?**	**Yes / No / Unsure**	**No → *Educational Assessment / OT*.**
	2. Does the child recognize familiar people or regular routines (e.g., meal, bath, nap time)?	**Yes / No / Unsure**	**No → *Psych / Early Childhood Development*.**

	3. Does the child try to use objects correctly (e.g., comb, cup, spoon)?	Yes / No / Unsure	No → *Educational / OT*.
Social / Behavioural	1. Does the child play beside other children or show interest in what others are doing?	Yes / No / Unsure	No → *Counselling / Early Childhood Intervention*.
	2. Does the child copy actions like waving, clapping, or dancing?	Yes / No / Unsure	No → *Psychosocial / OT*.
	3. Does the child show affection to caregivers (e.g., hugs, smiles, comes for comfort)?	Yes / No / Unsure	No → *Counselling / Early Childhood Support*.

Red Flags (Immediate Referral)

- **No single words by 18 months.**
- **Not walking independently by 18 months.**
- **Eyes not aligned or frequently bumping into objects.**
- **No response to name or familiar sounds.**
- **No interest in play or people.**
- **Loss of skills the child previously had (regression).**
- **Persistent stiffness or limpness in limbs.**

Age Band 3–4 Years

Domain	Question	Response Options	Referral Trigger / Red Flag
Hearing	1. Does the child respond when you call from another room or in a group setting?	Yes / No / Unsure	No → *Audiology / Hearing Block*.
	2. Does the child follow short stories or instructions without needing repetition?	Yes / No / Unsure	No → *Audiology / Speech & Language*.
	3. Does the child answer simple questions like “What is your name?” or “Where is mummy?”	Yes / No / Unsure	No → *Speech & Language / Audiology*.

Vision	**1. Does the child recognize familiar people or objects from a distance (e.g., parent, matatu, goat)?**	**Yes / No / Unsure**	**No → *Vision Block*.**
	2. Does the child avoid bright light or squint often when looking far?	**Yes / No / Unsure**	**Yes → *Vision Block* (possible eye sensitivity).**
	3. Does the child trip over obstacles or miss steps when walking or running?	**Yes / No / Unsure**	**Yes → *Vision Block / Physiotherapy*.**
Communication	**1. Does the child speak in short sentences of 3–4 words (e.g., "I want water")?**	**Yes / No / Unsure**	**No → *Speech & Language Therapy*.**
	2. Can the child be understood by familiar adults most of the time?	**Yes / No / Unsure**	**No → *Speech & Language*.**
	3. Does the child ask questions like "what" or "where"?	**Yes / No / Unsure**	**No → *Speech & Language / Cognitive Assessment*.**
Mobility – Gross	**1. Can the child run, climb small steps, and kick a ball?**	**Yes / No / Unsure**	**No → *Physiotherapy*.**
	2. Does the child jump with both feet or stand on one foot for a short time?	**Yes / No / Unsure**	**No → *Physiotherapy*.**
	3. Does the child move confidently around obstacles when playing?	**Yes / No / Unsure**	**No → *Physiotherapy / OT*.**
Mobility – Fine	**1. Can the child draw simple shapes (line, circle) or imitate them?**	**Yes / No / Unsure**	**No → *Occupational Therapy*.**
	2. Can the child hold a crayon or pencil between fingers (not fist)?	**Yes / No / Unsure**	**No → *Occupational Therapy / Early Learning*.**
	3. Can the child feed themselves with minimal spilling?	**Yes / No / Unsure**	**No → *OT / Feeding Therapy*.**

Cognition / Learning	1. Can the child name familiar objects or animals when asked?	Yes / No / Unsure	No → *Educational / Psychological Assessment*.
	2. Can the child match similar objects by color or shape (cup, toy, plate)?	Yes / No / Unsure	No → *Educational / OT*.
	3. Does the child remember simple routines (e.g., prayer, song, daily activities)?	Yes / No / Unsure	No → *Educational / Psych*.
Social / Behavioural	1. Does the child play with other children, sharing or taking turns?	Yes / No / Unsure	No → *Counselling / Psychosocial*.
	2. Does the child copy adult actions like sweeping, talking on the phone, or pretending to cook?	Yes / No / Unsure	No → *OT / Early Childhood Support*.
	3. Does the child show emotions (happy, sad, angry) appropriately for situations?	Yes / No / Unsure	No → *Counselling / Psychological*.

Red Flags (Immediate Referral)

- **Not speaking in short phrases by 3 years.**
- **Cannot follow two-step instructions.**
- **No interest in other children or pretend play.**
- **Stumbles or falls frequently.**
- **Cannot use hands for basic tasks (feeding, drawing).**
- **Eyes misaligned or frequent squinting.**
- **Does not respond to name or familiar voices.**

Age Band 5–6 Years

Domain	Question	Response Options	Referral Trigger / Red Flag
Hearing	1. Does the child hear well in a group or classroom without asking for repetition?	Yes / No / Unsure	No → *Audiology / Hearing Block*.

	2. Does the child respond when called softly from behind or another room?	**Yes / No / Unsure**	**No → *Audiology*.**
	3. Does the child enjoy listening to stories, songs, or conversations and respond appropriately?	**Yes / No / Unsure**	**No → *Audiology / Speech & Language*.**
Vision	**1. Does the child recognize friends, teachers, or vehicles from across the playground or road?**	**Yes / No / Unsure**	**No → *Vision Block*.**
	2. Does the child hold books or items too close to the face when looking?	**Yes / No / Unsure**	**Yes → *Vision Block*.**
	3. Does the child have frequent eye rubbing, blinking, or squinting?	**Yes / No / Unsure**	**Yes → *Vision Block*.**
Communication	**1. Does the child use full sentences and take part in simple conversations?**	**Yes / No / Unsure**	**No → *Speech & Language Therapy*.**
	2. Can unfamiliar adults understand the child most of the time?	**Yes / No / Unsure**	**No → *Speech & Language*.**
	3. Does the child tell simple stories or describe recent events clearly?	**Yes / No / Unsure**	**No → *Speech & Language / Educational Assessment*.**
Mobility – Gross	**1. Can the child run, hop, and skip without frequent falling?**	**Yes / No / Unsure**	**No → *Physiotherapy*.**
	2. Can the child climb stairs alternating feet without holding the rail?	**Yes / No / Unsure**	**No → *Physiotherapy / OT*.**
	3. Can the child throw and catch a medium-sized ball?	**Yes / No / Unsure**	**No → *Physiotherapy / OT*.**
Mobility – Fine	**1. Can the child draw a person, house, or familiar object with basic parts (e.g., head, legs)?**	**Yes / No / Unsure**	**No → *Occupational Therapy / Educational*.**

	2. Can the child use scissors or hold a pencil correctly for writing or drawing?	**Yes / No / Unsure**	**No → *Occupational Therapy*.**
	3. Can the child button clothes, zip a bag, or tie shoelaces?	**Yes / No / Unsure**	**No → *OT / Self-Care Skills Training*.**
Cognition / Learning	**1. Can the child name basic colors, numbers (1–10), and common shapes?**	**Yes / No / Unsure**	**No → *Educational / Psychological Assessment*.**
	2. Can the child match or sort items by color, size, or function (cup, spoon, plate)?	**Yes / No / Unsure**	**No → *Educational / OT*.**
	3. Can the child recall simple stories or routines from school/home?	**Yes / No / Unsure**	**No → *Educational / Psych*.**
Social / Behavioural	**1. Does the child take turns and share toys with others during play?**	**Yes / No / Unsure**	**No → *Counselling / Early Childhood Intervention*.**
	2. Does the child manage simple self-care tasks like dressing and toileting independently?	**Yes / No / Unsure**	**No → *OT / Counselling*.**
	3. Does the child show interest in playing with friends or joining group games?	**Yes / No / Unsure**	**No → *Counselling / Psychosocial*.**

Red Flags (Immediate Referral)

- **Not understandable to strangers by 5 years.**
- **Cannot hop, run, or climb stairs confidently.**
- **Cannot draw simple figures or hold a pencil properly.**
- **Avoids play with other children or prefers to stay alone.**
- **Does not follow two- to three-step instructions.**
- **Frequent squinting, eye rubbing, or difficulty recognizing faces.**
- **No clear hand preference by 6 years.**

Age Band 7–12 years (early to mid school-age)

Domain	Question	Response Options	Referral Trigger / Red Flag
Hearing	1. Does the child hear clearly in class without asking for repetition?	Yes / No / Unsure	No → Audiology / Hearing Block.
	2. Does the child turn up the TV or phone volume higher than others?	Yes / No / Unsure	Yes → Audiology.
	3. Does the child miss instructions or appear inattentive, though alert?	Yes / No / Unsure	Yes → Audiology / S&L review.
Vision	1. Does the child struggle to see writing on the blackboard from a normal distance?	Yes / No / Unsure	Yes → Vision Block.
	2. Does the child bring books very close when reading or writing?	Yes / No / Unsure	Yes → Vision Assessment.
	3. Does the child complain of frequent headaches or eye pain after reading?	Yes / No / Unsure	Yes → Vision Block.
Communication	1. Does the child speak clearly and use age-appropriate grammar and vocabulary?	Yes / No / Unsure	No → Speech & Language Therapy.
	2. Does the child understand the teacher's explanations and respond appropriately?	Yes / No / Unsure	No → S&L / Educational Assessment.
	3. Does the child participate in conversations with peers confidently?	Yes / No / Unsure	No → Counselling / S&L.
Mobility – Gross	1. Can the child run, jump, and participate in school games without difficulty?	Yes / No / Unsure	No → Physiotherapy.
	2. Does the child have any limp, joint pain, or stiffness limiting play?	Yes / No / Unsure	Yes → Physiotherapy.

	3. Does the child tire easily or struggle to climb stairs?	Yes / No / Unsure	Yes → Physiotherapy / Medical review.
Mobility – Fine / Coordination	1. Can the child write legibly for their grade level?	Yes / No / Unsure	No → Occupational Therapy.
	2. Does the child use both hands equally well, or prefer one hand for tasks?	Yes / No / Unsure	No clear preference by 8 yrs → OT / Neuro screen.
	3. Does the child handle daily tools (pencil, ruler, cup, spoon) without dropping them?	Yes / No / Unsure	No → OT.
Cognition / Learning	1. Is the child able to read and understand short paragraphs for their class?	Yes / No / Unsure	No → Educational Assessment / Psych.
	2. Does the child remember instructions and complete homework on time?	Yes / No / Unsure	No → Educational / Psychological.
	3. Does the child have trouble with number concepts, spelling, or writing?	Yes / No / Unsure	Yes → Educational Assessment / Learning Support.
Social / Behavioural	1. Does the child make and keep friends easily?	Yes / No / Unsure	No → Counselling / Psychosocial.
	2. Does the child show sudden mood changes, aggression, or withdrawal?	Yes / No / Unsure	Yes → Counselling / Mental-health referral.
	3. Does the child follow school and home rules appropriately for age?	Yes / No / Unsure	No → Counselling / Educational Psych.
Self-care / Daily Living	1. Can the child bathe, dress, and feed independently?	Yes / No / Unsure	No → OT / Functional Skills training.
	2. Does the child manage toileting and hygiene independently?	Yes / No / Unsure	No → OT / Counselling.

	3. Does the child organize personal items (books, uniform, bag) without help?	Yes / No / Unsure	No → Educational / OT.

Red Flags (Immediate Referral)

- **Difficulty seeing the board, hearing teachers, or following instructions.**
- **Speech not understood by peers or teachers.**
- **Frequent falling, limping, or avoiding games.**
- **Writing far below class level or frequent letter reversals beyond 8 years.**
- **Persistent withdrawal, aggression, or sadness.**
- **Regression (losing previously mastered academic or self-care skills).**

Age Band 13–17 Years

Domain	Question	Response Options	Referral Trigger / Red Flag
Hearing	1. Does the young person hear clearly during conversations or group activities without asking for repetition?	Yes / No / Unsure	No → Audiology / Hearing Service Point.
	2. Do they often misunderstand what is said or seem not to respond when spoken to?	Yes / No / Unsure	Yes → Audiology / Speech & Language.
	3. Have they had ear pain, discharge, or ringing (buzzing) sounds in the ear recently?	Yes / No / Unsure	Yes → Audiology / ENT review.
Vision	1. Does the young person have trouble seeing objects, reading, or identifying faces from a normal distance?	Yes / No / Unsure	Yes → Vision Service Point.
	2. Do they complain of frequent headaches, eye strain, or squinting when reading or doing close work?	Yes / No / Unsure	Yes → Vision / Medical review.

	3. Do they bump into objects, trip often, or prefer sitting very close when watching TV or using a phone?	**Yes / No / Unsure**	**Yes → Vision / Occupational Therapy.**
Communication	**1. Does the young person express themselves clearly and confidently when talking to others?**	**Yes / No / Unsure**	**No → Speech & Language / Counselling.**
	2. Do they understand what others say in normal conversation and respond appropriately?	**Yes / No / Unsure**	**No → Speech & Language / Educational Assessment.**
	3. Do they avoid speaking or show anxiety when asked to talk or present ideas?	**Yes / No / Unsure**	**Yes → Counselling / Psychosocial Support.**
Mobility – Gross	**1. Can the young person walk, run, or use stairs without support or visible strain?**	**Yes / No / Unsure**	**No → Physiotherapy / Occupational Therapy.**
	2. Do they report pain, stiffness, or fatigue when moving or doing chores?	**Yes / No / Unsure**	**Yes → Physiotherapy / Medical referral.**
	3. Do they use or need any mobility aid such as crutches, wheelchair, or prosthesis?	**Yes / No / Unsure**	**Yes → Physiotherapy / Assistive Technology Service Point.**
Mobility – Fine / Coordination	**1. Can the young person write, draw, or handle small tools easily?**	**Yes / No / Unsure**	**No → Occupational Therapy.**
	2. Do they perform self-care activities (washing, dressing, grooming) without assistance?	**Yes / No / Unsure**	**No → Occupational Therapy.**
	3. Do they drop things frequently or struggle with hand control?	**Yes / No / Unsure**	**Yes → Occupational Therapy.**
Cognition / Learning	**1. Does the young person remember new information, instructions, or routines easily?**	**Yes / No / Unsure**	**No → Educational / Psychological Assessment.**

	2. Do they find it difficult to stay focused during learning or daily tasks?	Yes / No / Unsure	Yes → Counselling / Educational Psychology.
	3. Have they shown a sudden drop in performance or loss of interest in learning, work, or play?	Yes / No / Unsure	Yes → Counselling / Psychological review.
Social / Behavioural	1. Does the young person make and maintain friendships or positive relationships?	Yes / No / Unsure	No → Counselling / Psychosocial Support.
	2. Have there been recent changes in behaviour such as isolation, anger, or risk-taking?	Yes / No / Unsure	Yes → Counselling / Mental Health referral.
	3. Has anyone expressed concern about their behaviour (e.g., skipping work/school, aggression, substance use)?	Yes / No / Unsure	Yes → Safeguarding / Counselling.
Self-care / Daily Living & Vocational Readiness	1. Can the young person manage personal care and home responsibilities independently?	Yes / No / Unsure	No → Occupational Therapy / Life Skills.
	2. Do they manage their own time, money, or belongings responsibly?	Yes / No / Unsure	No → Life Skills / Vocational Guidance.
	3. Are they interested in learning practical skills (e.g., tailoring, farming, ICT, carpentry)?	Yes / No / Unsure	No → Vocational Assessment / Career Guidance.

Red Flags (Immediate Attention or Priority Referral)

- **Sudden loss of hearing or vision.**
- **Withdrawal, sadness, self-harm talk, or aggression.**
- **Frequent falls, weakness, or pain in movement.**
- **Rapid decline in communication or learning ability.**
- **Neglect or abuse indicators, including visible injuries or fear.**
- **Use of drugs, alcohol, or unsafe relationships.**

Functional Screening – Adults (18 years and above)

Domain	Question (to be asked verbatim)	Response Options	Referral Trigger / Red Flag
Hearing	1. Do you hear clearly when people talk to you in a normal voice?	Yes / No / Not sure	"No" → Audiology / Hearing Service.
	2. Do you often ask people to repeat what they say?	Yes / No / Not sure	"Yes" → Audiology / Speech & Language.
	3. Do you have ear pain, discharge, or ringing sounds in your ears?	Yes / No / Not sure	"Yes" → Audiology / ENT.
Vision	1. Do you have trouble seeing people's faces or reading text, even with glasses?	Yes / No / Not sure	"Yes" → Vision Service.
	2. Do your eyes hurt, feel itchy, or do you get headaches when using your eyes?	Yes / No / Not sure	"Yes" → Vision / Medical.
	3. Do you move close to objects or the TV to see clearly?	Yes / No / Not sure	"Yes" → Vision / Occupational Therapy.
Communication	1. Do you find it easy to express yourself and make others understand you?	Yes / No / Not sure	"No" → Speech & Language Therapy.
	2. Have you noticed any change in how you speak (e.g., slurred or unclear words)?	Yes / No / Not sure	"Yes" → Speech & Language / Medical.
	3. Do you avoid talking with others because it's hard to communicate?	Yes / No / Not sure	"Yes" → Counselling / Psychosocial.
Mobility – Gross Movement	1. Can you walk and move around safely without support?	Yes / No / Not sure	"No" → Physiotherapy.

	2. Do you often fall, feel weak, or lose your balance when walking?	Yes / No / Not sure	"Yes" → Physiotherapy / Medical.
	3. Do you use a wheelchair, crutches, or walking stick?	Yes / No / Not sure	"Yes" → Physiotherapy / AT Service.
Mobility – Fine Motor (Hands/Coordination)	1. Can you hold small items like coins, pens, or keys easily?	Yes / No / Not sure	"No" → Occupational Therapy.
	2. Do your hands shake, feel weak, or numb?	Yes / No / Not sure	"Yes" → OT / Medical.
	3. Can you dress, eat, and wash yourself without help?	Yes / No / Not sure	"No" → Occupational Therapy.
Cognition / Memory / Thinking	1. Do you sometimes forget things like appointments or tasks?	Yes / No / Not sure	"Yes" → Psychology / Counselling.
	2. Do you have trouble focusing or making decisions?	Yes / No / Not sure	"Yes" → Psychology / Counselling.
	3. Have you noticed changes in how you think or remember recently?	Yes / No / Not sure	"Yes" → Psychology / Medical review.
Social / Behavioural / Emotional	1. Do you enjoy spending time with family and others?	Yes / No / Not sure	"No" → Counselling / Social Work.
	2. Do you often feel sad, anxious, or easily angered?	Yes / No / Not sure	"Yes" → Counselling / Mental Health.
	3. Have you been hurt, neglected, or felt unsafe at home or work?	Yes / No / Not sure	"Yes" → Safeguarding / Crisis Response.

Self-care & Daily Living	**1. Can you take care of yourself (bathing, dressing, meals)?**	**Yes / No / Not sure**	**"No" → OT / Support Services.**
	2. Do you need help to manage money, transport, or medication?	**Yes / No / Not sure**	**"Yes" → Social Work / OT.**
	3. Have you stopped doing activities you used to do easily?	**Yes / No / Not sure**	**"Yes" → OT / Physiotherapy.**
Work / Study / Productivity	**1. Are you currently working, studying, or doing income activities?**	**Yes / No / Not sure**	**"No" → Vocational / Rehabilitation Services.**
	2. Do you find your work or chores difficult because of a health or physical problem?	**Yes / No / Not sure**	**"Yes" → OT / Physio / Vocational.**
	3. Would you like support to learn new skills or start working again?	**Yes / No / Not sure**	**"Yes" → Vocational Rehabilitation / AT.**

Red Flags (Intake Officer to Refer Immediately)

- **Sudden hearing, vision, or movement loss.**
- **Severe sadness, hopelessness, or mention of suicide.**
- **Signs of violence, abuse, or neglect.**
- **Seizures, stroke, or fainting in recent weeks.**
- **Loss of ability to perform self-care.**

Client Type Logic: New editable; Old–New verification mode (read-only until **Enable Edit**).

H) Referral & Presenting Concern (concise)

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Referral Source	Multi-select	Self / School / Hospital / Community Worker / Social media / Court / Other	Multiple allowed.
Reason for Visit (one-liner)	Short text	—	**One concise line**; avoid narratives.

Reports Uploaded	File upload	PDF/JPG/PNG	Optional unless used to adjust verified data.

Client Type Logic: New editable; Old–New verification mode.

I) Service Screening, Posting & Availability

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Service Category (select all that apply)	Multi-select	Educational Assessment / Psychological Assessment / Audiology / Physiotherapy / Occupational Therapy / Speech & Language Therapy / Vision / Counselling / Assistive Technology consult / Nutrition / Other	Multi-select allowed.
Primary Service Posting (required)	Dropdown	List of KISE service points	**Required**; one primary.
Secondary / Cross-Posting (optional)	Multi-select	List of KISE service points	Optional; creates internal referral(s).
Service Availability (live)	Read-only	Available / Unavailable + Reason	Pulls from Customer Care daily status.
Next Available Slot (per service)	Read-only	Date / time / provider	Booking engine view.
Booking Request	Button	“Send to Booking Queue”	Creates booking request(s) for Customer Care.
Handover Note to Service	Short text	1–2 lines max	No narratives; key cue(s) only.

Client Type Logic: Same for New and Old–New. If service unavailable, proceed to **Deferral & Closure**.

J) Payment Pathway Preview (no billing here)

Field	Input Type	Drop Options / Source / Logic	Validation / Notes

Proposed Payment Pathway	Dropdown	Social Health Authority (SHA) / National Council for Persons with Disabilities (NCPWD) / eCitizen / Insurance / Out-of-Pocket	**Preview only**; final at Payment.
Eligibility Status (pre-check)	Read-only	Eligible / Unknown / Not eligible	Based on documents captured + rules of thumb.
Documents Required (if any)	Auto list	Per pathway	E.g., NCPWD card, SHA registration evidence.
Action	Button	"Refer to Payment Administrator"	Handoff to Payment module.

K) Deferral & Closure

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Deferral Needed?	Toggle	Yes / No	If No → proceed to Payment handoff.
Deferral Reason	Dropdown	Service unavailable / Missing documents / Client request / Other (specify)	Required if deferred.
Next Appointment Date	Date picker	—	Required if deferred.
Client Sensitized	Checkbox	Yes / No	Must be **Yes** to save deferral.
Notes to Client	Short text	—	Optional.
Create Check-Out (if deferral at Intake)	Button	Close visit at Intake	Logs reason + next date.

L) Time & Audit Fields

Field	Input Type	Drop Options / Source / Logic	Validation / Notes
Intake End Time	Auto	Set on "Complete Intake"	Duration computed.
Officer Signature / ID	Auto	From login	Audit trail.

Data Quality Banner	Read-only	“X required fields missing” + jump chips	Blocks completion until resolved.
Audit Trail Drawer	Read-only	Event log	Includes: intake.edit_started, intake.field_changed, intake.edit_saved, intake.verified_no_changes, intake.deferral_created.

Verification-Mode (Old–New) — Section Unlock Rules (recap)

- **Default:** Section is **read-only** if it already contains data.
- **Unlock:** Click **Enable Edit** → choose **Reason Code** (Admin correction / Evidence updated / Missing/Outdated data / Other).
- **Evidence:** If editing **Name / DOB / National ID / NCPWD Number**, the system **requires an attachment** before save (or admin override).
- **Unpopulated fields:** Always **editable** (no unlock needed).

System Behaviours (shared)

- **Autosave** every 60s; soft-lock per record; resume on same user.
- **Validation** both per-field and cross-field (e.g., DOB ↔ age; Kenyan phone pattern; NCPWD format if available).
- **Routing** after completion: **Status = “Cleared for Payment”** → auto-enqueue to Payment; create posting requests for Customer Care.
- **No duplication:** Merge rules maintain one central record per UCI.

ANNEX:

Rationale and References:

Rationale for Functional Screening at Intake

Purpose:
To enable early, objective identification of developmental, functional, and participation needs across age groups — guiding timely referral to appropriate service points (vision, hearing, therapy, education, psychosocial, or assistive technology).

1. Early Identification and Referral

Early screening allows timely detection of developmental delays, sensory loss, or functional limitations before they progress. WHO and UNICEF recommend early detection as a key strategy for inclusive child development and disability-responsive services.

2. Functional, Not Diagnostic

The questions are screening-based, not clinical assessments. They help non-specialist intake officers flag possible issues that warrant further review by specialists. This aligns with the WHO's *Developmental Difficulties in Early Childhood Framework* and the AAP's screening principles.

3. Age- and Context-Appropriate

Questions are simple, culturally adapted, and matched to developmental expectations per age band (0–6 years, 7–12 years, 13–17 years, adults). This ensures relevance for both school-going and non-school-going children, and supports inclusive service navigation.

4. Multidomain and Holistic

Domains (hearing, vision, communication, motor, cognitive/learning, social/behavioural, self-care) are consistent with the International Classification of Functioning, Disability and Health (ICF) and best practice in functional assessment.

5. Built-in Referral Logic

Each screening item links directly to a service point (e.g., "Yes → Vision Service Point") for efficient triage, avoiding duplication and ensuring continuity of care.

References (APA)

- World Health Organization. (2012). *Developmental Difficulties in Early Childhood: Prevention, Early Identification, Assessment and Intervention in Low- and Middle-Income Countries.* Geneva: WHO.
- UNICEF. (2021). *Early Childhood Development and Disability: A Discussion Paper.* New York: UNICEF.
- American Academy of Pediatrics. (2020). *Developmental Surveillance and Screening of Infants and Young Children. Pediatrics, 145*(1).
- Glascoe, F. P. (1991). Developmental screening: Rationale, methods, and application. *Infants & Young Children, 4*(1), 1–10.
- Early Childhood Technical Assistance Center (ECTA). (n.d.). *Screening and Evaluation: Infants and Toddlers—What and Why.* Retrieved from https://ectacenter.org/topics/earlyid/screeneval.asp
- World Health Organization. (2007). *International Classification of Functioning, Disability and Health: Children & Youth Version (ICF-CY).* Geneva: WHO.